

Supporting Early Psychosis

A BRIDGE guide for secondary schools and young adults: recognising early signs, responding safely, and keeping education on track.

Scope of this guide

This guide is written for Secondary and Young Adult settings specifically. Psychosis typically first emerges in adolescence or young adulthood, so this isn't adapted for younger children. It supports early recognition and consistent response. It is not a diagnostic or treatment tool, and any immediate safety concern should go to emergency services or a crisis line straight away (see Safety First, below).

Why This Matters

About 3 in every 100 people will experience psychosis at some point in their life, usually beginning in adolescence or young adulthood (Schiffman et al., 2018). Early, coordinated support (sometimes called Coordinated Specialty Care internationally, and delivered in Australia through services like Orygen and headspace's early psychosis programs) consistently improves outcomes and helps young people stay engaged in school and life more broadly. Schools sit in a genuinely powerful position here: the earlier the gap between first symptoms and effective care closes, the better the outcome tends to be.

Psychosis rarely announces itself with dramatic symptoms. It usually begins gradually: withdrawal, distrust, subtle misperceptions, which makes it easy to miss and easy for a young person to hide, particularly given the stigma still attached to the word.

Recognising the Early Signs

No single sign below means psychosis on its own. Most are things any teenager might show occasionally. What matters is a persistent pattern, especially one that's new, exaggerated, or starting to affect friendships and schoolwork.

- Perception and thinking: hearing, seeing, or sensing things others don't; unusual or fixed beliefs; speech that's hard to follow or rambling
- Withdrawal and functioning: spending far more time alone, stepping back from friends or hobbies, a real drop in grades or motivation
- Emotional changes: flat or unusually blunted reactions, or emotion that doesn't match the situation; extreme, unexplained fear
- Everyday changes: shifts in sleep or eating, a decline in self-care, trouble concentrating that's new for that student

If these things begin affecting friendships or schoolwork, not just showing up once, it's worth a closer look, not a wait-and-see approach.

What To Do When You're Concerned

- Escalate early, not after certainty: speak to a school counsellor, psychologist, or nurse; you don't need to be sure, you need to be observant
- Be thoughtful raising it with families: it can land as a shock, given home behaviour often looks different from what you're seeing at school
- Stay culturally aware: some beliefs that look unusual out of context (for example, beliefs about spirits) are ordinary within a particular family's culture or faith, not a symptom
- Involve the young person's mental health provider once one is engaged, so home, school, and treatment are working from the same picture

Safety First

If you're worried about immediate risk

Ask directly and sensitively about suicidal thoughts, actions, or plans. Asking does not put the idea there, and avoiding the question doesn't make the risk go away. Escalate immediately to a school-based health professional,

and don't leave a young person alone if you believe risk is immediate. In Australia: Lifeline 13 11 14, Kids Helpline 1800 55 1800, or 000 in an emergency.

People experiencing psychosis are far more likely to be the target of bullying or violence than the source of it, which is worth remembering and worth actively countering if you see it happening.

Reducing Stigma

Words casually borrowed from mental illness (“crazy,” “psychotic,” “bipolar” used to describe an ordinary bad mood) quietly teach a whole room what those words “really” mean, and it's usually wrong. Precise language (“that student seems upset”) costs nothing and keeps the door open for a student who might actually need to talk about a real symptom.

Supporting Their Education

In an Australian setting, this typically sits under reasonable adjustments through the Disability Standards for Education, documented via a Personalised Learning and Support Plan or the school's NCCD process, not a US-style IEP, but the same underlying idea: individualised, documented, and reviewed.

- A quiet space to work or sit an exam, away from noise or distraction
- Flexible deadlines, since symptoms genuinely ebb and flow
- Alternatives to public speaking tasks: recorded or one-on-one presentation instead
- Class notes or recordings if concentration is affected
- A clear, agreed “safe place” the student can go to during the school day if needed

A thoughtful transition plan matters most around any absence for treatment, built with the family and provider before the student returns, not assembled on their first day back. Returning after a hospital admission in particular can carry real shame; a warm, low-key welcome does more than any formal accommodation.

Hope & Recovery

Most young people with psychosis don't have symptoms that stay severe. They tend to ebb and flow, and recovery should be the expectation with the right support, not the exception. Young people experiencing psychosis are, in every other respect, exactly like their peers: same stresses, same friendships, same hopes for what comes next. That's the frame worth holding onto, for the student and for the adults around them.

How This Ties to BRIDGE

	BRIDGE step	Applied to early psychosis
B	Behaviour Has a Reason	Withdrawal, altered speech, or a grade drop is information, not defiance, the same lens applied to a more serious presentation.
R	Record the Pattern	Noticing when changes started and how they're progressing is what turns a vague worry into something a professional can act on quickly.
I	Identify One Shared Goal	Home, school, and any treatment provider working from the same picture, not three separate ones.
D	Design Matching Strategies	Accommodations that hold steady across every classroom and setting, not just the one where a teacher happened to notice.
G	Give Consistent Language	Precise, non-stigmatising language used the same way by every adult around the student.
E	Evaluate & Adjust Together	Regular review as symptoms ebb and flow: what helped last term may need to change this term.

Filling the Gap

Specialist assessment and early psychosis programs can carry long waits in Australia too, the same as the other spaces this pathway covers. You don't need a diagnosis to start recognising patterns, keeping language consistent, and putting simple classroom supports in place. That groundwork matters regardless of where a family is in the process of getting formal care.

References

Schiffman, J., Hoover, S., Roemer, C., Redman, S., & Bostic, J. (2018). Supporting Students Experiencing Early Psychosis in Middle School and High School. Alexandria, VA: National Association of State Mental Health Program Directors. Prepared for SAMHSA.

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